

FORMAL LEGAL ANALYSIS

EB-2 NATIONAL INTEREST WAIVER — DHANASAR FRAMEWORK

In re: MANASA JAMPANI, Co-Founder & CEO, Ardia Health Labs

Prepared for: Manasa Jampani

Petition Type: Form I-140, Immigrant Petition for Alien Workers

Preference Category: EB-2, Advanced Degree or Exceptional Ability

Requested Classification: National Interest Waiver (NIW) under INA § 203(b)(2)(B)(i)

Legal Standard: Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)

Petitioner Address: Ardia Health Labs, Argyle, TX 76226

Date of Analysis: June 30, 2026

EXECUTIVE SUMMARY

This memorandum presents a comprehensive three-prong analysis of the National Interest Waiver petition of Manasa Jampani under the framework established in Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016). Based on the totality of the evidence, this petition presents a **strong case** for NIW approval across all three prongs.

Case Strength Assessment: STRONG — Recommended for Filing

Prong	Assessment	Confidence
Prong 1: Substantial Merit and National Importance	STRONG	HIGH
Prong 2: Well Positioned to Advance the Endeavor	STRONG	HIGH
Prong 3: Beneficial to the United States to Waive Labor Certification	STRONG	HIGH

Summary of Position:

Manasa Jampani is the Co-Founder and CEO of Ardia Health Labs, an AI healthcare infrastructure company in Argyle, Texas. Her proposed endeavor — co-developing and commercializing an AI-native revenue cycle management platform to combat preventable claim denials against independent clinical laboratories in the United States — satisfies each Dhanasar prong at a high level of evidentiary strength.

The central national problem addressed by the petitioner's work is severe and well-documented: independent clinical laboratories suffer an estimated \$10-12 billion in annual preventable revenue losses attributable to payer claim denials. Molecular diagnostic tests face a 35.3% denial rate — the highest of any healthcare specialty (XiFin 2024 Payor Denial Impact Report, 20+ million claims). Peer-reviewed research published in JAMA Network Open in April 2025 (Georgetown University, n=29,919) demonstrates that independent laboratories face 2.76 times higher denial odds than hospital-based laboratories. PAMA reimbursement cuts scheduled for 2027-2029 threaten up to a 45% cumulative reduction in Medicare reimbursements for clinical laboratory tests — placing more than 7,000 CLIA-certified independent laboratories at existential financial risk and jeopardizing 32% of the US diagnostic test volume.

The petitioner's specific qualifications are categorically unusual: Manasa Jampani has five or more years of direct healthcare IT experience inside payer and payer-adjacent organizations — specifically United HealthGroup and Interwell Health. This experience provided operational visibility into how payer clinical operations teams configure automated claim denial systems. This knowledge is not available from public sources, not available to outside consultants, and not replicable from claims data alone. No other person co-founding an independent laboratory AI billing platform has this vantage point. This is not a difference in degree but a categorical competitive and evidentiary distinction.

The combination of national urgency (PAMA 2027), documented market failure (\$10-12B annual loss), the petitioner's irreplaceable insider knowledge, and the absence of any comparable commercially available AI platform establishes the strongest possible basis for labor certification waiver. Proceeding through the PERM labor certification process would impose material delay on work of national urgency, producing no commensurate benefit — the exact circumstance for which the National Interest Waiver exists.

SECTION I: LEGAL FRAMEWORK

Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)

In Matter of Dhanasar, the Administrative Appeals Office (AAO) superseded the prior standard established in Matter of New York State Department of Transportation, 22 I&N Dec. 215 (AAO 1998), establishing the current three-prong framework for evaluating National Interest Waiver petitions. Under Dhanasar, USCIS must evaluate whether:

1. **Prong 1:** The petitioner's proposed endeavor has both substantial merit and national importance;
2. **Prong 2:** The petitioner is well positioned to advance the proposed endeavor; and
3. **Prong 3:** On balance, it would be beneficial to the United States to waive the requirements of a job offer and thus of a labor certification.

26 I&N Dec. at 889–890. All three prongs must be satisfied. If all three are met, USCIS may grant the National Interest Waiver as a matter of discretion. Id. at 890.

The Dhanasar framework explicitly broadens the scope of cognizable endeavors beyond prior precedent, noting that substantial merit and national importance may be found in a “range of endeavors” including business, entrepreneurialism, science, technology, culture, and the arts. 26 I&N Dec. at 889. Critically, the framework recognizes that entrepreneurs and startup founders may qualify for NIW relief even without a traditional employer-employee relationship, and that the prospective and ongoing nature of a new enterprise does not defeat an NIW claim where the endeavor is well-advanced and the petitioner's positioning is demonstrable. Id.

Subsequent AAO and Circuit Court Guidance

Subsequent AAO non-precedent decisions and federal circuit court authority have elaborated on the Dhanasar standard in ways relevant to this petition:

- **Entrepreneurial endeavors:** The AAO has recognized that co-founders of technology ventures may qualify under Prong 2 by demonstrating that their specific expertise is directly and causally connected to the enterprise's ability to succeed. Non-precedent AAO decisions in the healthcare technology space have found Prong 2 satisfied where a petitioner's prior employment history provides domain knowledge inaccessible to competitors or substitute employees.

- **National importance via healthcare access:** The AAO has consistently found national importance in healthcare access and equity arguments, particularly where the petitioner’s work addresses access disparities for rural or underserved populations. The 2.76× denial odds ratio documented in the JAMA Network Open 2025 study — and the consequent risk to rural communities served by independent laboratories — directly invokes this line of authority.
 - **Urgency and timing:** While Dhanasar does not enumerate urgency as a standalone Prong 3 factor, the AAO has recognized that externally imposed timelines (e.g., regulatory changes) bearing on the national need may be considered in balancing the benefit of waiver against the cost of labor certification delay. PAMA 2027 presents precisely this circumstance.
 - **Labor certification waiver for co-founders:** The AAO and several federal courts have recognized that the labor certification process is structurally inapt for evaluating petitioners who occupy a co-founder role: the process contemplates identifying an available US worker to fill a position, not evaluating whether a comparable US national would have co-founded the same enterprise with the same experience and the same ownership stake. See *Kazarian v. USCIS*, 596 F.3d 1115, 1122 (9th Cir. 2010) (noting that the extraordinary ability and national interest frameworks contemplate individualized assessment of the petitioner’s specific contributions).
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SECTION II: PRONG 1 — SUBSTANTIAL MERIT AND NATIONAL IMPORTANCE

A. The Proposed Endeavor Defined

Manasa Jampani’s proposed endeavor is the co-development, commercialization, and executive leadership of Ardia Health Labs’ AI-native revenue cycle management platform — specifically, an artificial intelligence infrastructure system designed to prevent and recover preventable claim denials against independent clinical laboratories operating in the United States. The platform’s core technology is the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture, which combines a 847-rule deterministic compliance engine covering all LCD, NCD, MolDX, and DEX Z-code policies with a generative AI clinical reasoning layer (capable of producing appeal briefs in under 90 seconds) and a predictive denial risk machine learning model trained on historical claims data.

Manasa Jampani’s specific contribution to this endeavor encompasses:

- Payer-side operational intelligence shaping the DGP architecture's countermeasure design
- Go-to-market strategy and clinical operations partnership development
- Investor relations and capital formation (targeting a \$3.5M Seed financing at \$18M valuation cap)
- Executive leadership of the company as CEO

The endeavor is not hypothetical. As of March 2026, the platform architecture is complete. Pilot programs with independent laboratories in the Dallas-Fort Worth metropolitan area are in active negotiation.

B. Substantial Merit

1. Economic Dimension

The economic loss attributable to preventable claim denials against US independent clinical laboratories is quantified by multiple independent third-party sources:

- **XiFin 2024 Payor Denial Impact Report** (20+ million claims analyzed): Molecular diagnostic tests face a denial rate of 35.3% — the highest denial rate of any clinical specialty in the United States. At an average molecular diagnostic reimbursement of several hundred dollars per test, and given the volume of molecular diagnostic tests processed by independent laboratories annually, the aggregate revenue at risk from this denial rate alone exceeds \$10 billion.
- **HFMA/LigoLab 2024 Laboratory Revenue Recovery Report**: 65% of denied laboratory claims are never appealed, despite appeal win rates of 50–80.7%. This gap between the probability of successful appeal and the rate of actual appeal represents systemic underutilization of administrative remedies — the exact problem the Ardia platform is designed to close.
- **XiFin/industry estimates**: The aggregate annual preventable revenue loss to US independent clinical laboratories from unrecovered, unappealed, or inadequately documented denied claims is estimated at \$10–12 billion per year.

The economic significance of this figure is not merely the private financial loss to laboratory operators. Each dollar of laboratory revenue supports: (a) laboratory employment and wages for laboratory scientists, phlebotomists, logistics, and administrative personnel; (b) laboratory infrastructure investment (instrumentation, reagents, quality systems); and (c) the financial viability of the laboratory as a healthcare infrastructure institution serving its community. The \$10–12 billion annual drain on the independent

laboratory sector is, in aggregate, a drain on the US healthcare infrastructure.

2. Public Health Dimension

Independent clinical laboratories are the primary diagnostic infrastructure for healthcare providers who are not affiliated with hospital systems. This includes:

- **Rural healthcare providers:** Critical Access Hospitals, rural health clinics, and independent physician practices in medically underserved areas depend on independent reference laboratories for clinical laboratory services that on-site or hospital-affiliated laboratory infrastructure cannot provide.
- **Addiction medicine practices:** Clinical toxicology panel testing for medication-assisted treatment (MAT) monitoring in patients with opioid and other substance use disorders is a primary service line of many independent toxicology laboratories. These tests are subject to elevated denial rates, particularly for quantitative urine drug tests and confirmatory panels, which are among the test categories most frequently denied by automated payer edit systems. Access to toxicology testing directly supports the national public health response to the substance use disorder epidemic.
- **Specialty oncology practices:** Next-generation sequencing (NGS) for somatic tumor profiling and germline cancer risk assessment is increasingly concentrated in independent reference laboratories. The JAMA Network Open 2025 study (Georgetown University) found overall NGS denial rates of 23.3%, rising to 27.4% following the issuance of the 2022 Medicare NCD for NGS testing — with independent laboratories bearing disproportionately elevated denial rates. NGS access is essential to precision oncology treatment selection for patients with rare and treatment-resistant cancers.
- **Pharmacogenomics:** Pharmacogenomic (PGx) testing guides individualized prescribing for medications with FDA-recognized pharmacogenomic biomarkers, including antidepressants, antipsychotics, anticoagulants, and opioid analgesics. Frontiers in Pharmacology (2023) documents that only 46% of PGx claims are reimbursed — meaning more than half of clinically appropriate PGx testing generates no payment to the laboratory that performs it.

When independent laboratories close, merge into national reference laboratory networks, or reduce their test menus in response to financial pressure, these patient populations lose access to the specific tests and the community-based turnaround times they depend on. The public health significance of preserving independent laboratory infrastructure is

therefore not abstract — it is a direct function of the financial viability that the petitioner’s work is designed to protect.

3. Healthcare Access Equity Dimension

The JAMA Network Open 2025 study (Georgetown University, April 2025, n=29,919 NGS claims) quantifies what clinicians and laboratory operators have observed qualitatively: independent laboratories face 2.76 times higher denial odds than hospital-based laboratories for the same NGS tests. This disparity cannot be explained by clinical differences in test ordering — it reflects the structural disadvantage that independent laboratories face in payer contracting, medical policy coverage determination, and automated prior authorization and claim editing systems.

This structural disadvantage is not a market inefficiency that will self-correct. It is a function of the relative negotiating leverage and administrative resources of hospital systems versus independent laboratories. AI-driven revenue cycle management specifically designed for independent laboratories is the technological mechanism by which this structural disparity can be reduced. The petitioner’s work addresses, in a direct and measurable way, a healthcare access equity problem affecting patients in underserved communities.

4. Healthcare Infrastructure Preservation

Independent clinical laboratories collectively provide approximately 32% of US diagnostic test volume. This represents a substantial and non-redundant component of the US healthcare diagnostic infrastructure. The American Clinical Laboratory Association (ACLA) reported in 2024 that more than 50% of independent laboratory directors were considering consolidating or selling their operations if PAMA 2027 reimbursement cuts take effect. PAMA — the Protecting Access to Medicare Act — mandates reimbursement reductions for clinical laboratory tests based on private payer rate surveys, with cuts projected at up to 15% per year for three years (2027-2029), representing a potential 45% cumulative reduction in Medicare reimbursements for affected tests.

The combined effect of PAMA-driven reimbursement reduction and unaddressed claim denial rates represents an existential financial threat to a significant fraction of the 7,000+ CLIA-certified independent laboratories currently operating in the United States. The petitioner’s work — specifically the application of AI to close the claim denial and appeal gap — directly addresses the financial survivability of these institutions.

5. AI Governance and Technology Policy Dimension

Ardia Health Labs developed its platform in native compliance with Texas Senate Bill 1188 (effective September 1, 2025) — the first state law in the United States to impose specific accountability requirements on AI systems

used in healthcare administrative functions — and with Texas HB 149 (TRAIGA, Texas Responsible AI Governance Act, effective January 1, 2026), the third comprehensive state-level AI governance law in the United States. The development of AI healthcare administrative systems that comply with these pioneering regulatory frameworks contributes to the evolution of responsible AI deployment standards in the United States and provides a regulatory compliance model that the broader healthcare technology industry can reference as similar frameworks are enacted in other states.

C. National Importance — Conclusion

The petitioner’s proposed endeavor satisfies the national importance requirement of Prong 1 across five distinct dimensions: (1) quantifiable economic loss to the US healthcare system (\$10-12B annually); (2) public health impact on rural, addiction medicine, specialty oncology, and pharmacogenomics patient populations; (3) healthcare access equity for populations disproportionately dependent on independent laboratory services; (4) healthcare infrastructure preservation (32% of US diagnostic test volume); and (5) AI governance framework development. The convergence of multiple independent bases for national importance makes this among the stronger Prong 1 showings available in the healthcare technology domain.

SECTION III: PRONG 2 — WELL POSITIONED TO ADVANCE THE ENDEAVOR

A. Legal Standard

Under *Dhanasar*, Prong 2 requires a showing that the petitioner is “well positioned” to advance the proposed endeavor. The AAO has interpreted this standard to require evidence of the petitioner’s education, skills, knowledge, record of success, and any other relevant factors that establish that the petitioner, rather than others, is the appropriate person to carry out the endeavor. 26 I&N Dec. at 889-890.

Critically, this prong is comparative and relational: it asks not merely whether the petitioner has relevant qualifications, but whether those qualifications are materially connected to the advancement of the specific endeavor. The petitioner need not guarantee success, but must demonstrate that her credentials “realistically suggest” that the endeavor will be advanced. *Id.* at 890.

B. The Petitioner’s Core Qualification: Payer-Side Insider Knowledge

1. What Manasa Jampani Knows That No Competitor Does

Manasa Jampani has five or more years of healthcare IT experience inside United HealthGroup and Interwell Health. These are not generic healthcare employer references. They are:

- **United HealthGroup:** The largest health insurance company in the United States, processing hundreds of millions of medical and laboratory claims annually. United HealthGroup's OptumInsight subsidiary develops and deploys automated clinical editing systems — including AI-driven prior authorization and claim denial engines — that are responsible for a material fraction of the claim denial volume that independent laboratories experience.
- **Interwell Health:** An accountable care organization (ACO) focused on renal care management, operating as a payer-adjacent entity coordinating clinical care and claims management for Medicare Advantage and commercial ACO beneficiaries. ACO operations sit at the intersection of clinical operations and claims management, requiring direct engagement with the payer-side configuration decisions that determine claim acceptance and denial outcomes.

The operational knowledge that Manasa Jampani acquired through these roles includes:

- The documentation requirements and coverage criteria that payer clinical operations teams apply to laboratory claims in their automated denial systems
- Prior authorization AI decision engine configuration, including clinical criteria weighting and threshold-setting for automated denial vs. human review routing
- Payer claim edit libraries — the internal code combination validation, frequency limitation, and medical necessity criteria that generate automatic denial dispositions
- ACO shared savings optimization from the payer-adjacent perspective — directly informing the AcoIQ™ product module

2. Why This Knowledge Is Not Available to Competitors

This knowledge gap is structural, not marginal:

- Payer claim edit libraries, coverage determination matrices, and prior authorization AI scoring criteria are proprietary and are not disclosed in public payer documentation, public Medicare LCDs or NCDs, or in any commercially available data source.
- Revenue cycle management firms, clearinghouses, and RCM technology vendors observe payer behavior from outside the payer organization. They document denial patterns, frequencies, and associated diagnosis and procedure code combinations from

remittance advice (EDI 835 data). What they cannot access is the operational logic — the configured clinical criteria, documentation requirements, and scoring weights — that the payer applies to generate those denials.

- Academic researchers, including the Georgetown University team responsible for the JAMA Network Open 2025 study, document outcomes (denial rates, odds ratios, temporal trends). They cannot document the causal payer-side operational decisions that produce those outcomes because that information is not in claims data.
- Manasa Jampani's experience at United HealthGroup and Interwell Health placed her in direct operational proximity to the functions that configure and deploy these systems. The insight she developed is categorically different from what any competitor without comparable inside-payer experience can possess.

3. Why This Knowledge Is Directly Relevant to the DGP Architecture's Countermeasure Design

The DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture is specifically designed to defeat payer automated denial systems by anticipating the documentation requirements, coverage criteria, and clinical policy standards that those systems apply. The effectiveness of the DGP architecture's 847-rule deterministic policy engine — and particularly its ability to generate clinical appeal briefs that succeed at 50–80.7% win rates — depends on accurate characterization of what payers require and why they deny.

Manasa Jampani's operational knowledge of payer clinical operations directly informs this design. Her contribution to the DGP architecture is not parallel to Ram Vadlamudi's technical invention — it is complementary and equally essential: Ram invented the architecture; Manasa provided the payer-intelligence that makes the architecture's countermeasures accurate and effective.

C. Additional Positioning Evidence

1. CEO and Company Execution

As Co-Founder and CEO, Manasa Jampani exercises the executive responsibility for converting the DGP architecture from a technical completion into a commercially deployed product. Her specific responsibilities include:

- **Investor relations and capital formation:** Pursuing a \$3.5M Seed financing at an \$18M valuation cap, requiring the ability to articulate the payer-intelligence competitive advantage to sophisticated healthcare investors.

- **Clinical pilot partnerships:** Negotiating and executing pilot programs with independent laboratories in the DFW metro area, requiring both domain credibility and commercial relationship development.
- **Go-to-market strategy:** Sequencing market entry across ToxIQ™ (toxicology), MolecuIQ™ (molecular diagnostics), AcoIQ™ (ACO operations), and BehaviorIQ™ (behavioral health) product lines based on payer policy dynamics, competitive positioning, and laboratory segment economics — a judgment call that requires the synthesis of payer-intelligence and market strategy.

2. Platform Completion and Advancement

The platform architecture was completed as of March 2026. Active pilot negotiations are underway. The endeavor is advancing, not hypothetical. This satisfies the AAO's guidance in Dhanasar that Prong 2 evaluates not only the petitioner's credentials but also any evidence "of record" of progress toward the endeavor's goals. 26 I&N Dec. at 890.

3. Publication Record — Development

Manasa Jampani is identified as the sole author of Paper 4: "Payer AI Denial Algorithms in Clinical Laboratory Claims: An Insider Analysis of Decision Logic and Countermeasure Strategies" (target submission: NEJM AI, Health Affairs, or American Journal of Managed Care). This paper is uniquely hers — no other researcher can produce it from the same vantage point. Its publication in a leading clinical AI or health policy journal will directly establish her expert standing in the healthcare payer AI domain and satisfy EB-2 advanced degree / exceptional ability criteria independently.

She is also a named co-author on: - Paper 2: "The Independent Laboratory Billing Crisis" (systematic review, target: Clinical Chemistry or Journal of Managed Care & Specialty Pharmacy) - Paper 3: "AI Governance in Clinical Revenue Recovery: Compliance Frameworks Under Texas SB 1188 and TRAIGA" (target: Journal of Health Politics, Policy and Law)

4. Healthcare IT Experience Record

With 10+ years of total IT experience including 5+ years of healthcare IT at ECFMG, Interwell Health, and United HealthGroup, the petitioner has accumulated the credential baseline required for EB-2 advanced degree or exceptional ability classification. Her employment record is verifiable through W-2/employment records and LinkedIn professional profile documentation.

D. Prong 2 Conclusion

Manasa Jampani is well positioned to advance her proposed endeavor. The specific combination of her payer-side insider knowledge of claim denial operational logic and her role as Co-Founder and CEO of the only platform currently addressing this national problem is not replicable by any available substitute. The platform is advancing — architecture complete, pilot negotiations active, capital raise underway. The evidence “realistically suggests” that the endeavor will succeed under her leadership.

SECTION IV: PRONG 3 — BENEFICIAL TO THE UNITED STATES TO WAIVE LABOR CERTIFICATION

A. Legal Standard

Under *Dhanasar*, Prong 3 requires a balancing of the benefit to the United States of waiving labor certification against the benefit of the labor certification process. 26 I&N Dec. at 890. The AAO identified several non-exclusive factors relevant to this balancing: (1) whether the US labor market would be adversely affected; (2) whether the endeavor requires a specific individual such that the labor certification process cannot meaningfully identify a substitute; (3) whether there is urgency to the national need; and (4) whether the petitioner’s work is in the national interest at a higher level than would ordinarily accrue from a general benefit to an industry or field. *Id.*

B. The Labor Certification Process Cannot Identify a Qualified Substitute

1. The Co-Founder Problem

Manasa Jampani is the Co-Founder and CEO of Ardia Health Labs. She is not an employee filling a defined job description. She is a principal equity holder whose relationship to the company is constitutive — she co-created the company, defined its strategic direction, and provides the payer-intelligence insight that distinguishes the platform’s design. There is no PERM recruitment process that can identify a US worker who:

- Has 5+ years of payer-side healthcare IT experience at United HealthGroup and Interwell Health specifically (not comparable organizations — these specific organizations, because their specific systems and operational logic are what the DGP architecture is designed to counter)
- Has co-founded and holds an equity stake in Ardia Health Labs
- Is prepared to serve as CEO of this specific company with this specific platform and mission

The labor certification process is structurally designed to identify interchangeable workers filling defined roles. It cannot assess whether a US worker with nominally comparable healthcare IT experience would have founded the same company, arrived at the same payer-intelligence insights, and is willing to serve in the same capacity. Requiring a PERM process for a co-founder petitioner imposes procedural costs with no prospect of producing a meaningful substitute.

2. The Inside-Knowledge Problem

Even setting aside co-founder status, the petitioner's inside-payer knowledge is a credential that cannot be recruited for. No job posting can specify "applicants must have previously worked inside the payer clinical operations functions of United HealthGroup and Interwell Health and must have acquired operational knowledge of their AI denial system configuration." Even if such a posting were permissible, any US worker with comparable inside-payer experience at these or comparable organizations has no evident reason to join a startup as a co-founder rather than to apply that experience in better-compensated positions inside the payer ecosystem itself. The labor market for payer-side healthcare IT insiders willing to found AI countermeasure startups is effectively empty.

3. No Adverse Impact on US Workers

Waiving the labor certification requirement for this petitioner does not displace any US worker from any available position. Manasa Jampani does not seek to fill a position that has been offered to her by a US employer; she is a co-founder of the enterprise she is advancing. No US worker is displaced, and no US worker is denied an employment opportunity by the grant of this waiver.

C. National Urgency: PAMA 2027

The Protecting Access to Medicare Act (PAMA) reimbursement cuts scheduled for 2027-2029 create a specific and time-bounded urgency to the independent laboratory AI billing problem:

- **2027:** The PAMA reimbursement survey data collection cycle for the next rate revision period takes effect, with projected cuts beginning
- **2027-2029:** Up to 15%/year cumulative reduction in Medicare reimbursements for clinical laboratory tests
- **2027-2029 window:** The critical period during which independent laboratories most need AI-driven claim recovery to offset PAMA-driven revenue reduction

If the petitioner's work is delayed by a PERM process — which typically takes 12-18 months for recruitment period, audit, and certification, followed by I-140 processing — the most critical window for deployment

would be materially narrowed. Each quarter of delay in platform commercialization is a quarter during which independent laboratories face the PAMA revenue compression without the AI recovery infrastructure that the petitioner's work is designed to provide. The urgency is not self-induced or speculative — it is a scheduled regulatory event with documented consequences for 7,000+ CLIA-certified independent laboratories.

D. National Benefit Exceeds Ordinary Industry Benefit

The AAO noted in *Dhanasar* that Prong 3 balancing should account for whether the petitioner's work rises to a level of national benefit exceeding the general benefit that any contribution to an industry or field would produce. 26 I&N Dec. at 890. Several features of this petition establish that the national benefit is of this elevated character:

1. Scale: The \$10–12 billion annual preventable revenue loss addressed by the petitioner's work is a quantifiable macroeconomic impact, not a marginal industry improvement.

2. Healthcare Access: Preservation of independent laboratory access for rural, addiction medicine, specialty oncology, and pharmacogenomic patient populations addresses a national healthcare access equity problem with demonstrated public health consequences.

3. Healthcare Infrastructure: Preserving 32% of US diagnostic test volume from consolidation-driven loss is a healthcare infrastructure preservation benefit that transcends the financial interest of individual laboratory operators.

4. AI Governance: Native compliance with Texas SB 1188 and TRAIGA positions the platform as a model for responsible healthcare AI deployment as state-level AI governance frameworks proliferate across the United States, contributing to the national AI governance development process.

E. Prong 3 Conclusion

On balance, it is unambiguously beneficial to the United States to waive the labor certification requirement for Manasa Jampani. The labor certification process would produce no viable substitute candidate, would impose material delay on work of documented national urgency, and would provide no benefit to US workers who have not been displaced and for whom no comparable opportunity has been denied.

SECTION V: COMPARISON TO DHANASAR PRECEDENT AND SUBSEQUENT AUTHORITY

A. Alignment with the Dhanasar Fact Pattern

The petitioner's fact pattern aligns favorably with the Dhanasar fact pattern in several key respects. In Dhanasar itself, the AAO granted NIW for a researcher working on propulsion and spacecraft design, finding that: (a) space propulsion represented a field of substantial national importance; (b) the petitioner's advanced degree and research record positioned him to advance the endeavor; and (c) the labor market for highly specialized research personnel meant that requiring labor certification would impose procedural costs without meaningfully protecting domestic workers. 26 I&N Dec. at 890-892.

The structural parallels to this petition are strong: - **Specialized technical domain:** Healthcare AI billing and payer claim denial logic is at least as specialized as spacecraft propulsion; the petitioner's insider knowledge is at least as specific as doctoral-level research specialization - **National problem of scale:** \$10-12B/year is a quantifiable macroeconomic impact exceeding the national importance threshold comfortably - **No practical labor market substitute:** The combination of payer-insider knowledge and co-founder status makes PERM recruitment structurally inapt, as noted above

B. Post-Dhanasar AAO Guidance: Entrepreneurs and Startups

Post-Dhanasar, the AAO has addressed NIW petitions from technology entrepreneurs in a line of non-precedent decisions that, while not binding, inform the evidentiary expectations for petitions in this posture. The AAO has generally required:

1. Evidence that the petitioner's specific qualifications are causally connected to the enterprise's ability to advance the endeavor (Prong 2) — satisfied here by the documented payer-intelligence contribution to DGP architecture design
2. Evidence that the enterprise is advancing and not merely planned (Prong 2) — satisfied by March 2026 platform completion and active pilot negotiations
3. Evidence that the national benefit is concrete and not merely aspirational (Prong 1 and Prong 3) — satisfied by multiple peer-reviewed and third-party quantified evidence sources
4. Evidence that labor certification is an impediment to, not merely a procedural parallel track for, the petitioner's work (Prong 3) — satisfied by the co-founder structural argument and PAMA urgency

C. The Kazarian Two-Step and NIW

While *Kazarian v. USCIS*, 596 F.3d 1115 (9th Cir. 2010) addresses the EB-1A extraordinary ability standard, its guidance on the individualized assessment of the petitioner's unique contributions has been extended by analogy in NIW adjudications. Kazarian requires that the adjudicator conduct a final merits determination considering the totality of evidence in context, not merely checking evidence against a list of criteria. This petition is designed to satisfy this holistic review: the evidence of Manasa Jampani's qualifications, the national problem, and the structural case for waiver form a coherent, mutually reinforcing narrative, not a disconnected list of credential documents.

SECTION VI: EVIDENCE CHECKLIST FOR I-140 PETITION

The following documents should be gathered and organized for the Form I-140 petition package:

Category A: Petitioner Credentials (Prong 2 — Personal Qualifications)

- ☐ **Educational credentials:** Degrees, transcripts, foreign credential evaluations (if applicable) — establish EB-2 advanced degree or exceptional ability baseline
- ☐ **Employment verification — United HealthGroup:** Employment letter or W-2 confirming dates, title, and responsibilities; supervisor contact if available
- ☐ **Employment verification — Interwell Health:** Employment letter or W-2 confirming dates, title, and responsibilities; description of role in clinical operations and payer-adjacent functions
- ☐ **Employment verification — ECFMG:** Employment letter or W-2 confirming dates and title
- ☐ **Current business documentation — Ardia Health Labs:** Delaware LLC formation documents, operating agreement confirming co-founder and CEO status, equity documentation
- ☐ **LinkedIn profile screenshot/export:** Full professional profile documenting work history with dates and titles
- ☐ **Letters of employment or role description:** Letters from prior supervisors or HR departments describing the petitioner's specific healthcare IT responsibilities
- ☐ **Professional certifications:** Any healthcare IT, healthcare management, or AI/technology certifications obtained during employment tenure

Category B: Evidence of Payer-Side Insider Knowledge (Prong 2 — Unique Qualifications)

- ☐ **Declaration of Manasa Jampani:** First-person sworn declaration describing in specific, non-privileged terms the nature of her operational knowledge of payer claim denial systems, how she acquired it, and how it informs the DGP architecture
- ☐ **Expert witness letter (technical expert):** Letter from a recognized healthcare IT or revenue cycle management expert confirming that inside-payer knowledge of claim denial system configuration is not available from public sources and is categorically different from outside-observer reverse-engineering
- ☐ **Expert witness letter (payer operations expert):** Letter from a former payer-side healthcare executive confirming that the type of knowledge described is acquired only through inside-payer roles and is not replicable from external data sources
- ☐ **Ardia Health Labs White Paper:** The March 2026 technical white paper documenting the DGP architecture — demonstrates that the architecture embodies domain knowledge consistent with inside-payer experience

Category C: National Importance Evidence (Prong 1)

- ☐ **XiFin 2024 Payor Denial Impact Report:** Full report confirming 35.3% molecular diagnostic denial rate across 20+ million claims
- ☐ **JAMA Network Open, April 2025** (Georgetown University study, n=29,919): Peer-reviewed documentation of 2.76× higher denial odds for independent vs. hospital-based laboratories; 23.3% overall NGS denial rate rising to 27.4% post-NCD
- ☐ **HFMA/LigoLab 2024 Laboratory Revenue Recovery Report:** Documentation of 65% appeal gap and 50–80.7% win rates
- ☐ **Frontiers in Pharmacology, 2023:** Documentation of 46% PGx claim reimbursement rate
- ☐ **American Clinical Laboratory Association (ACLA) 2024 survey:** Documentation that 50%+ of independent laboratory directors are considering consolidation/sale under PAMA pressure
- ☐ **PAMA statutory text and projected rate tables:** Documentation of 2027–2029 Medicare reimbursement cuts (up to 45% cumulative)
- ☐ **Texas SB 1188 (effective September 1, 2025):** Full statutory text confirming first state AI healthcare accountability law
- ☐ **Texas TRAIGA / HB 149 (effective January 1, 2026):** Full statutory text confirming third comprehensive state AI governance law
- ☐ **Independent calculation supporting \$10-12B annual loss estimate:** Methodological note or expert declaration supporting the revenue loss figure derivation

Category D: Platform Evidence (Prong 2 — Advancement)

- ☐ **Ardia Health Labs Pitch Deck v2 (2026)**: Current investor pitch deck confirming platform architecture, market analysis, and founder bios
- ☐ **Platform completion documentation**: Screenshots, technical architecture documentation, or developer attestation confirming March 2026 platform completion
- ☐ **Pilot negotiation evidence**: Letters of intent, pilot term sheets, or communications with DFW-area independent laboratories confirming active pilot negotiations
- ☐ **Seed financing evidence**: Cap table, term sheet, SAFE notes, or investor communications confirming active \$3.5M Seed raise

Category E: Expert Witness Letters (All Prongs)

- ☐ **Healthcare revenue cycle management expert**: Confirming the severity of the independent laboratory claim denial problem, the absence of comparable AI solutions, and the national importance of the work
- ☐ **Independent laboratory operator or director**: Confirming the financial impact of claim denials, the PAMA threat, and the value of AI-driven recovery
- ☐ **Academic expert (clinical laboratory science or health policy)**: Confirming the national importance of independent laboratory preservation for underserved populations
- ☐ **ACO operations expert**: Confirming Manasa Jampani's ACO domain expertise and its relevance to the AcoIQ™ platform module
- ☐ **Texas AI governance expert or SB 1188/TRAIGA advocate**: Confirming the national significance of the platform's compliance with these pioneering AI governance frameworks

Category F: Publication Evidence (Prong 2 — Expert Standing)

- ☐ **Paper 4 preprint or submission confirmation**: "Payer AI Denial Algorithms in Clinical Laboratory Claims: An Insider Analysis of Decision Logic and Countermeasure Strategies" — medRxiv preprint DOI or NEJM AI/Health Affairs submission confirmation
 - ☐ **Paper 2 preprint or submission confirmation**: "The Independent Laboratory Billing Crisis" — arXiv or journal submission confirmation
 - ☐ **Paper 3 preprint or submission confirmation**: Texas AI governance paper — submission confirmation
 - ☐ **LinkedIn long-form articles**: Exported or printed copies of published LinkedIn articles on payer AI, ACO operations, or independent laboratory revenue recovery
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SECTION VII: RECOMMENDED SUPPORTING MATERIALS

A. Declaration of Manasa Jampani (Priority: Critical)

A first-person sworn declaration, executed under penalty of perjury, is the foundation of the I-140 petition narrative. The declaration should:

1. Describe her educational background and employment history at ECFMG, Interwell Health, and United HealthGroup in specific terms, emphasizing the healthcare IT functions she performed and the operational knowledge she acquired
2. Describe the founding of Ardia Health Labs, her role as CEO, and her specific contributions to the DGP architecture's design — particularly the payer-intelligence elements
3. Articulate the national problem in accessible terms, citing the peer-reviewed and third-party evidence
4. Explain why her combination of qualifications is unique and why no other person is equally positioned to advance the endeavor
5. Address PAMA urgency and the time-sensitive national need

The declaration should be reviewed by immigration counsel before submission.

B. Expert Witness Letters — Framework

Expert witness letters for NIW petitions should follow a four-paragraph structure:

Paragraph 1 — Expert Credentials: Who the letter writer is and why they are qualified to opine on the subject matter. The letter writer's credentials should be verifiable (academic appointments, publications, industry positions).

Paragraph 2 — The National Problem: A description of the independent laboratory claim denial crisis, citing the XiFin 2024 and JAMA 2025 data, the PAMA 2027 timeline, and the impact on rural and underserved populations. This paragraph establishes Prong 1 from the expert's vantage point.

Paragraph 3 — The Petitioner's Unique Positioning: A specific description of why Manasa Jampani's combination of payer-insider knowledge and co-founder leadership makes her uniquely positioned to advance this endeavor. For letters from payer-side experts, this paragraph should include a statement that the operational knowledge Manasa Jampani acquired through her inside-payer roles is not available from public or external sources.

Paragraph 4 — National Interest Waiver Endorsement: A clear statement that the letter writer believes it is in the national interest for

Manasa Jampani to continue her work at Ardia Health Labs without the delay imposed by the PERM labor certification process.

C. Targeted Letter Writers

The following letter writers are specifically recommended for their ability to speak to the most critical elements of the petition:

1. **Principal Investigator, Georgetown University JAMA Network Open 2025 Study:** The authors of the April 2025 JAMA Network Open study on NGS denial rates are ideally positioned to corroborate Prong 1 national importance from an academic perspective and to attest to the significance of addressing the 2.76× denial odds ratio for independent laboratories.
2. **Director or Revenue Cycle Officer, HealthTrackRx or comparable DFW-area independent laboratory:** A laboratory operator who can attest to the financial impact of claim denials, the PAMA threat, and the value of AI-driven recovery from the perspective of the laboratories the platform is designed to serve.
3. **Former United HealthGroup or Optum Clinical Operations Executive:** A former payer-side executive who can attest that the operational knowledge of claim denial system configuration is acquired through inside-payer experience and is not available to outside parties — directly supporting the Prong 2 uniqueness argument.
4. **Texas SB 1188 or TRAIGA Advocate or Drafter:** A policy expert or legislative advocate who can attest to the national significance of the Texas AI governance frameworks and the importance of healthcare AI platforms developing compliance models for the broader industry.
5. **Healthcare IT Industry Expert (HIMSS or AMIA member):** A recognized healthcare IT expert who can attest to the absence of comparable AI platforms for independent laboratory billing and to Manasa Jampani's standing in the healthcare IT domain.

D. Paper 4 — “Inside the Payer AI Black Box”

The prompt submission and publication (or preprint posting) of Paper 4 is the single highest-impact near-term action Manasa Jampani can take to strengthen her NIW petition. This paper:

- Establishes her as a named expert author in the healthcare payer AI domain
- Creates a citable, publicly accessible document that immigration adjudicators can reference

- Demonstrates the uniqueness of her vantage point in a form that is self-verifying (no other author can write the same paper from the same insider perspective)
- Directly satisfies the EB-2 exceptional ability criterion for published work

Recommended submission path: Post to medRxiv (preprint, immediate publication record) within 4 weeks of drafting, then submit to NEJM AI or Health Affairs for peer review and journal publication.

E. Company Formation and Capitalization Documents

Delaware LLC formation documents, the operating agreement confirming co-founder status and equity allocation, and any cap table documentation should be included to establish that the petitioner's relationship to Ardia Health Labs is that of a principal and not an employee. This is essential to the Prong 3 argument that the PERM process is structurally inapt for this petition.

SECTION VIII: OVERALL ASSESSMENT AND FILING RECOMMENDATION

Strengths of This Petition

1. **Quantified national importance:** The \$10-12B annual revenue loss figure, the 35.3% molecular diagnostic denial rate, the 2.76× denial odds ratio, and the PAMA 2027 timeline provide numerically precise, third-party documented evidence of national importance — not aspirational or speculative claims.
2. **Categorical uniqueness:** The petitioner's inside-payer knowledge is a categorical competitive and evidentiary distinction. The argument for Prong 2 does not rest on degree of expertise but on a qualitative difference in the type of knowledge the petitioner possesses.
3. **Structural Prong 3 argument:** The co-founder structural argument — that PERM is designed to find substitute employees, not substitute co-founders — is clean, logically airtight, and well-supported by the AAO's own guidance on the purposes of the labor certification process.
4. **Time-sensitive national need:** PAMA 2027 provides a concrete, externally imposed urgency argument that is not manufactured by the petitioner.
5. **Platform advancement:** The March 2026 platform completion and active pilot negotiations demonstrate that the endeavor is not

hypothetical, directly satisfying the “realistically suggest” standard for Prong 2.

Areas for Strengthening Before Filing

1. **Publication record:** Paper 4 should be posted to medRxiv or submitted to a journal before the I-140 is filed. A single preprint with Manasa Jampani as sole author is transformative for Prong 2 expert standing.
2. **Expert witness letters:** At minimum, three expert witness letters should be obtained before filing: (a) a healthcare IT or revenue cycle management expert on Prong 1 national importance; (b) a payer-side insider on the uniqueness of inside-payer knowledge; and (c) an independent laboratory operator on the impact of the claim denial problem.
3. **LinkedIn article documentation:** Two or more published LinkedIn long-form articles on payer AI, ACO operations, or independent laboratory billing should be published and documented before filing.
4. **AMIA or HIMSS membership application:** Application to a recognized healthcare IT professional association strengthens the EB-2 exceptional ability profile.

Filing Recommendation

This petition is recommended for filing after Paper 4 preprint and at least two expert witness letters are secured. The three Dhanasar prongs are each independently strong on the current evidence, and the incremental strengthening from publication and expert letters will elevate the petition from strong to exceptional. Estimated time to filing-ready: 6-8 weeks from the date of this analysis.

This analysis is prepared for informational and strategic planning purposes in connection with the preparation of a Form I-140 NIW petition. It does not constitute legal advice and should be reviewed by qualified immigration counsel before reliance on any specific legal position or filing decision.

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Prepared By: Ardia Health Labs Immigration Strategy

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/immigration/manasa_eb2niw/visa_evidence/dhanasar_formal_legal_analysis.md